2nd Chance Survey: Preliminary Report

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Report Purpose

This report presents preliminary descriptive findings from the 2nd Chance Survey, examining the demographic characteristics, professional training, geographic distribution, and clinical practice settings of plastic and reconstructive surgeons currently practicing in Sub-Saharan Africa (SSA). The analysis characterizes the surveyed workforce and identifies patterns in professional backgrounds, institutional affiliations, and geographic distribution that may inform further research and workforce planning in the region.

Findings reflect the participating respondents and the facilities they reported and should not be interpreted as representative of the entire plastic and reconstructive surgery workforce in SSA.

Response Rate and Data Completeness

Potential participants were identified through three complementary sources. The COSECSA Entire Surgeon Membership List (n = 1,655) and the COSECSA Plastic Reconstructive Surgeon Fellow List (n = 129) constituted the COSECSA stream. The 2nd Chance Professional Network (n = 57) and 2nd Chance COSECSA Conference Attendance list (n = 40) were merged into a combined list of 97 individuals. Online searches of published literature and other sources identified a further 35 individuals. After deduplication — 33 duplicates from the 2nd Chance stream and 3 from online sources (36 total removed) — these streams yielded 225 unique potential participants (THIS NUMBER IS YET TO BE CONFIRMED WITH COSECSA).

Of the 51 individuals who accessed the survey, 9 were excluded: 1 declined informed consent and 8 were not currently practicing in SSA, leaving 42 eligible participants (82.4% of those who accessed the survey). The response rate relative to the deduplicated pool was 18.7% (42/225).

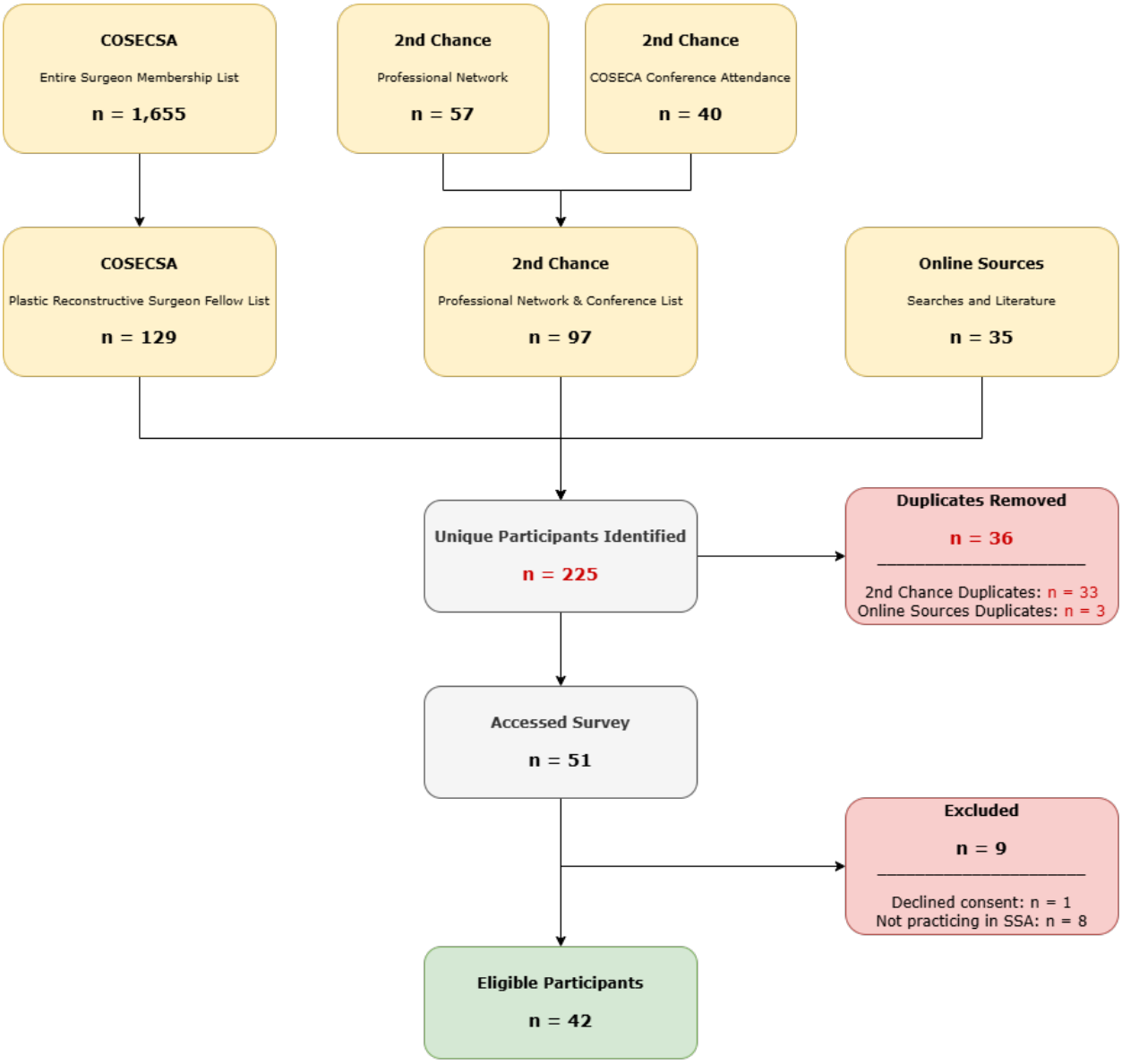


Figure 1: Figure 1. Participant identification and inclusion flowdiagram. Numbers highlighted in red are to be confirmed with COSECSA

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During data cleaning, one respondent aged 50–59 years reported a specialty accreditation year of 1950, implying 76 years since accreditation — inconsistent with the reported age and not reliably correctable. Both the accreditation year and the derived years-in-practice variable were treated as missing for this individual, who was retained in all other analyses for which valid data were available. Analyses involving accreditation year therefore have a denominator of N = 41.

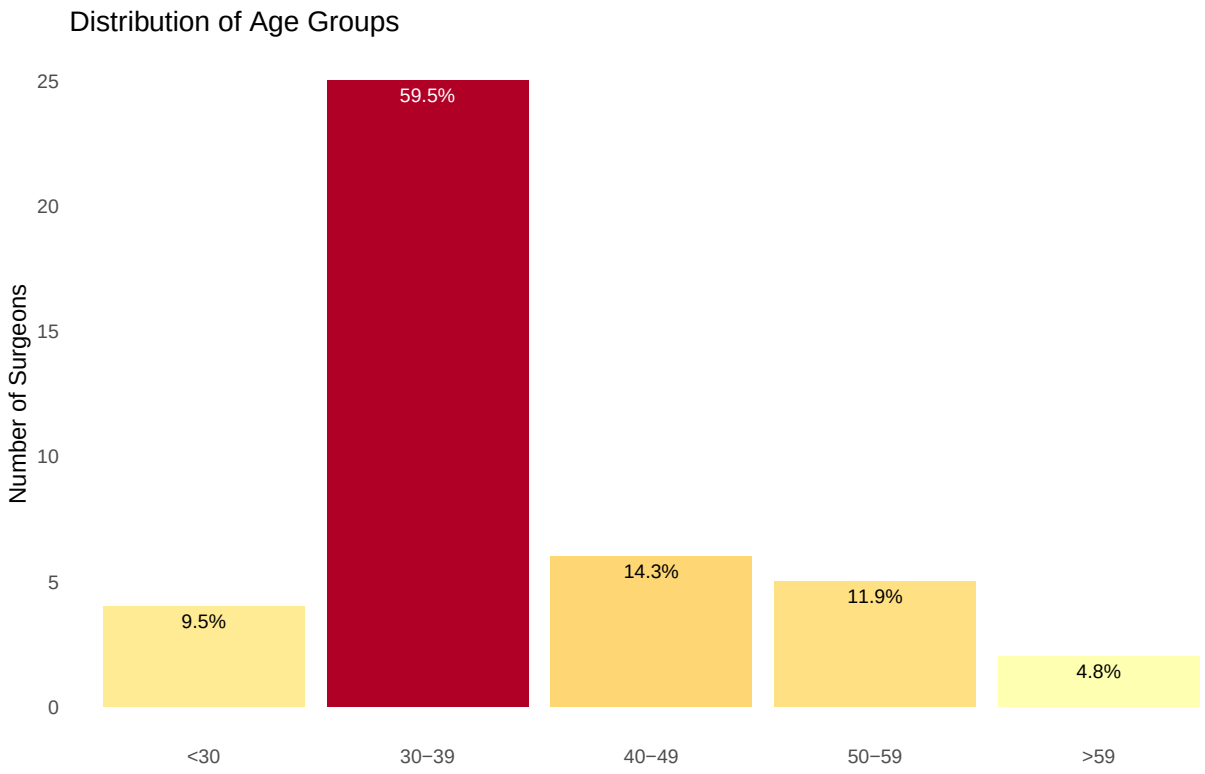
Respondent Characteristics

Age and Gender

The demographic composition of survey respondents was examined according to age group and gender. Together, these characteristics provide an overview of the age and gender representation within the participating surgical workforce.

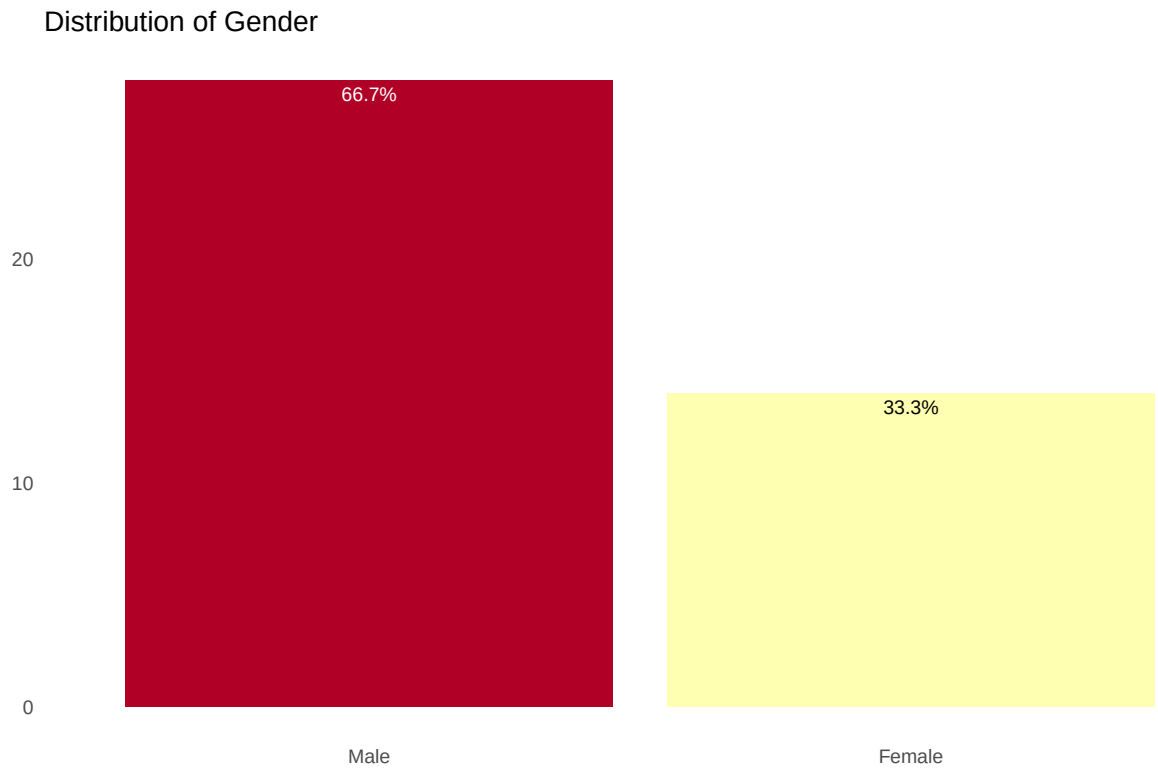
Age Distribution

Respondents represented multiple age groups, indicating variation in career stages among participating surgeons. The age distribution is presented in Figure 1, highlighting the relative representation of surgeons across age categories.



Respondents skewed markedly young: the largest age group was 30–39 years, comprising 25 of 42 participants (59.5%). Surgeons aged 40–49 accounted for 6 (14.3%), those aged 50–59 for 5 (11.9%), those under 30 for 4 (9.5%), and those over 59 for 2 (4.8%). Nearly three-quarters of respondents (74 of every 100) were therefore under 40 years of age, reflecting a predominantly early- to mid-career workforce.

Gender Distribution



The sample was predominantly male: 28 respondents (66.7%) identified as male and 14 (33.3%) as female. This gender imbalance is consistent with broader patterns in surgical specialties globally, though the relatively substantial proportion of female surgeons (one in three) is noteworthy for the SSA context and merits monitoring in future surveys.

Participant Characteristics

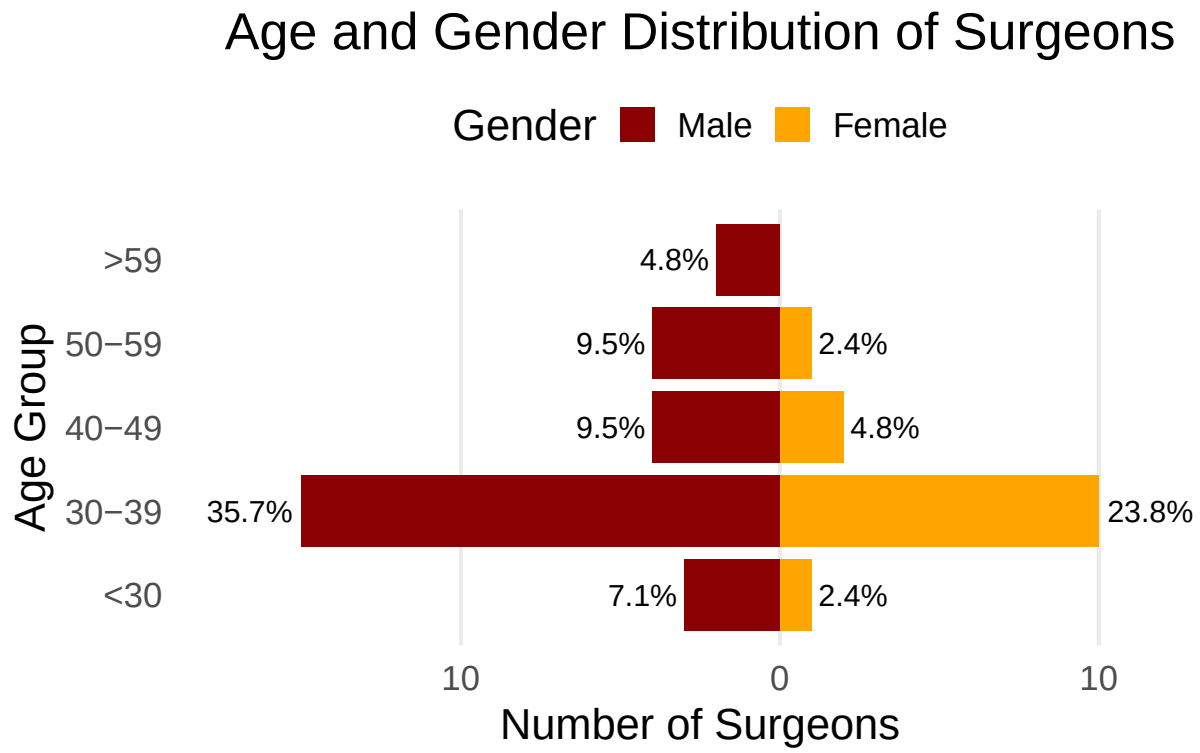
Table 1 summarizes respondents' demographic characteristics and countries of practice. Age, gender, and geographic distributions are reported as frequencies and percentages, with denominators based on available responses for each characteristic.

Age and Gender Distribution

The combined age and gender distribution is presented in Figure 3. This analysis examines how age groups are represented among male and female respondents, providing additional insight into the demographic composition of participating surgeons.

Participant Characteristics
N = 42 participants

Patients (age groups), n (%)	**N = 42 participants**
30-39	25 (59.5%)
40-49	6 (14.3%)
50-59	5 (11.9%)
<30	4 (9.5%)
>59	2 (4.8%)
Patients Gender, n (%)	**N = 42 participants**
Male	28 (66.7%)
Female	14 (33.3%)
Country of Practice, n (%)	**N = 42 participants**
Ethiopia	14 (33.3%)
Rwanda	9 (21.4%)
Kenya	4 (9.5%)
Uganda	4 (9.5%)
Democratic Republic of the Congo	2 (4.8%)
Nigeria	2 (4.8%)
Tanzania	2 (4.8%)
Burundi	1 (2.4%)
Côte d'Ivoire	1 (2.4%)
Malawi	1 (2.4%)
Somalia	1 (2.4%)
Togo	1 (2.4%)



Geographic Distribution of Plastic Surgery Practice

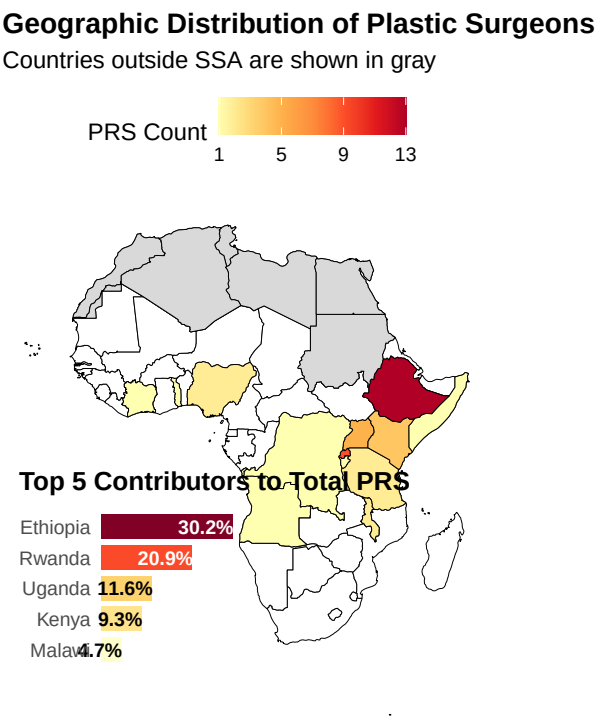
Reported countries of practice among survey respondents

Country	Reported practices, n (%)
Ethiopia	13 (30.2%)
Rwanda	9 (20.9%)
Uganda	5 (11.6%)
Kenya	4 (9.3%)
Malawi	2 (4.7%)
Nigeria	2 (4.7%)
Tanzania	2 (4.7%)
Angola	1 (2.3%)
Burundi	1 (2.3%)
Congo - Kinshasa	1 (2.3%)
Côte d'Ivoire	1 (2.3%)
Somalia	1 (2.3%)
Togo	1 (2.3%)

Note: Respondents could report practicing in multiple countries. Percentages are based on the total number of country mentions (N = 43), not unique respondents.

Country of Practice

The table below summarizes the geographic distribution of reported countries of practice among respondents, including the frequency and percentage of each country. As respondents could practice in multiple countries, percentages are based on the total number of country mentions.



Respondents reported practicing plastic surgery across multiple countries in sub-Saharan Africa (SSA). Ethiopia accounted for the largest proportion of reported countries of practice, with 13 mentions (30.2%), followed by Rwanda with 9 (20.9%), Uganda with 5 (11.6%), and Kenya with 4 (9.3%). Malawi contributed

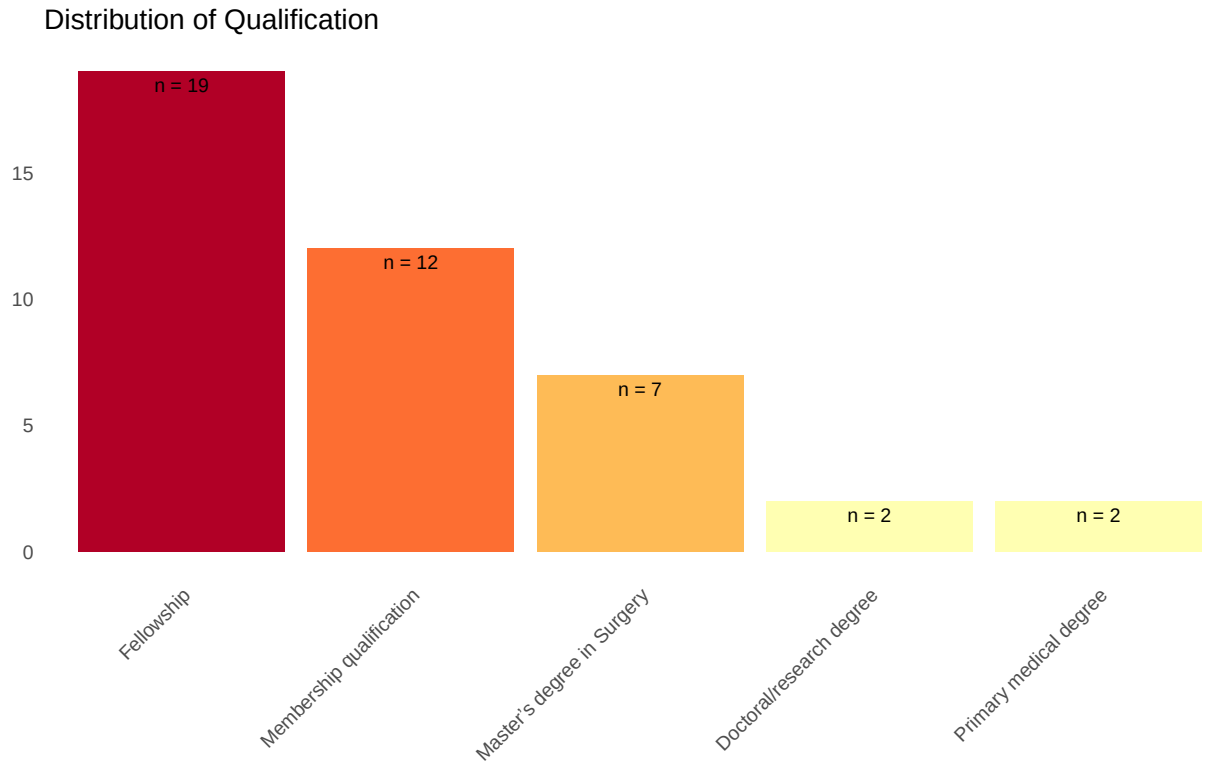
2 mentions (4.7%). Together, these five countries accounted for 33 of the 43 reported country mentions (76.7%).

The geographic distribution of reported practice locations was concentrated in East Africa, with Ethiopia and Rwanda alone accounting for 51.2% of all country mentions. Because respondents could report practicing in more than one country, these percentages represent the distribution of reported practice locations rather than the proportion of unique respondents. This concentration may reflect the geographic reach of survey recruitment networks, particularly COSECSA, and should not be interpreted as representative of the overall distribution of plastic surgeons or surgical capacity across SSA.

Professional Training

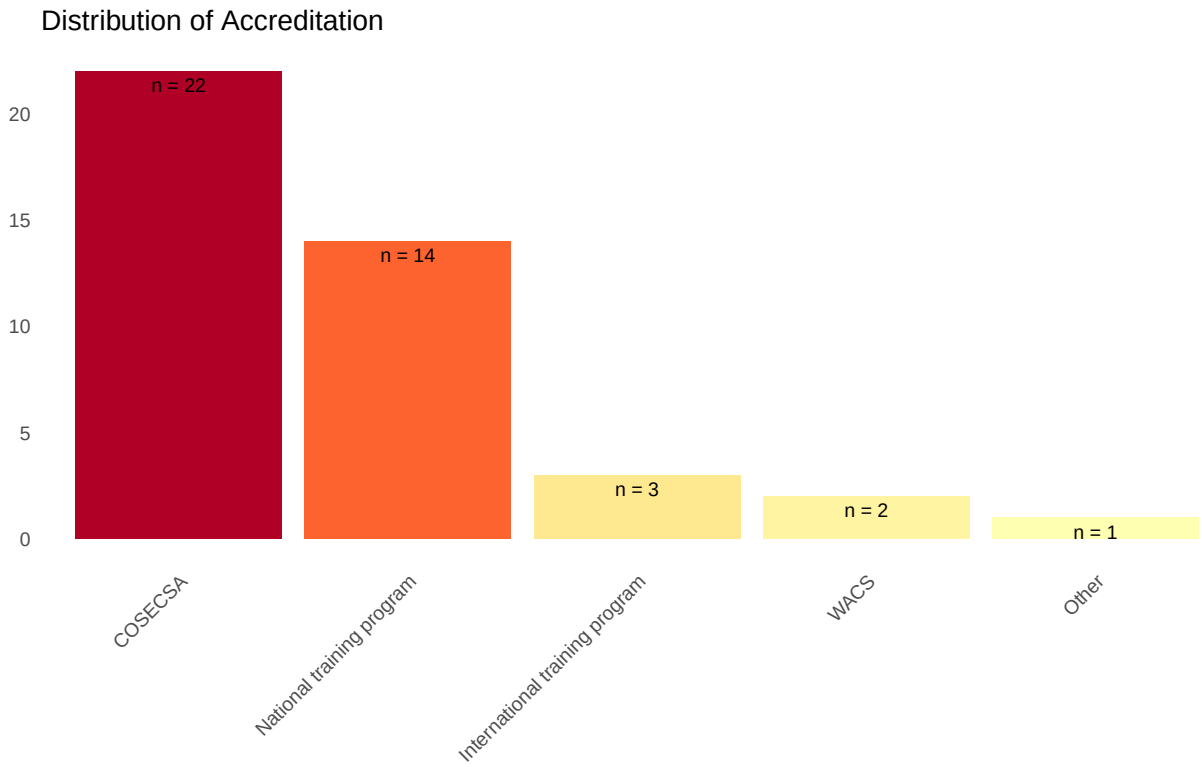
Professional training characteristics were examined to describe the educational qualifications, accreditation pathways, years since specialty accreditation, and current professional positions of participating surgeons.

Highest Qualification



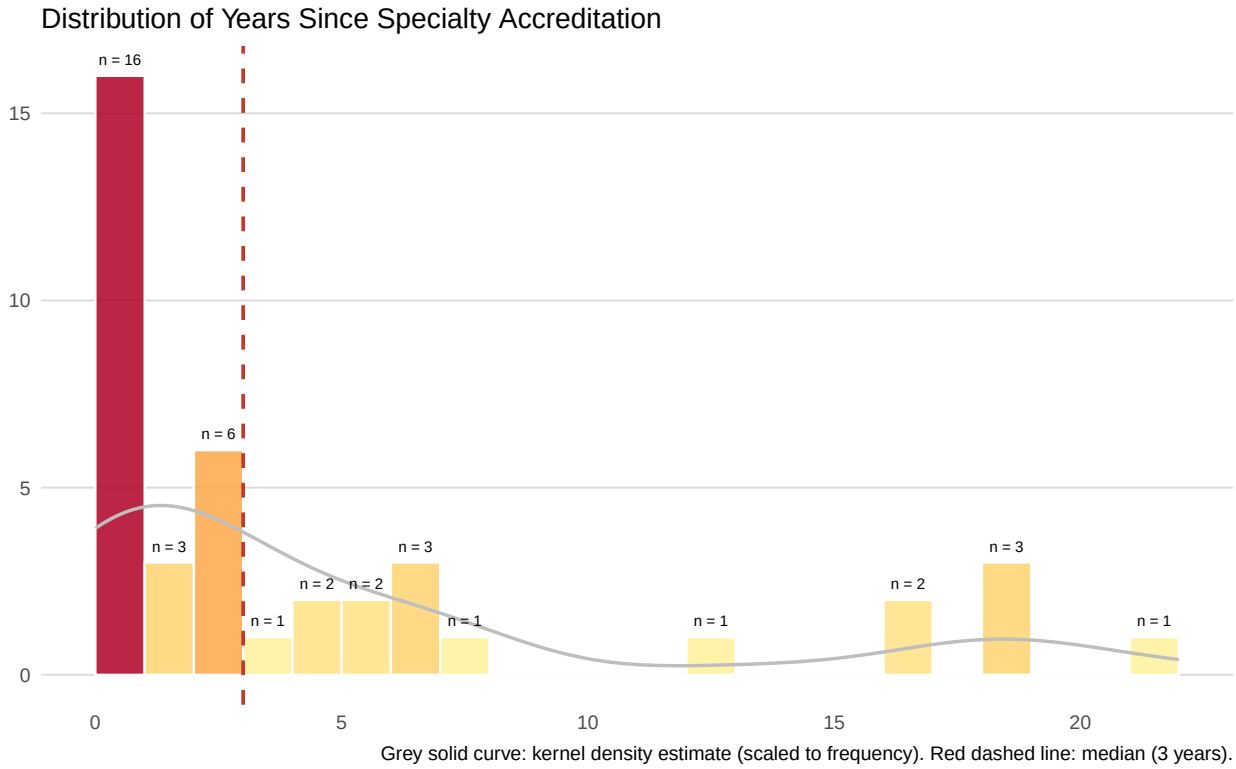
The most common highest qualification was a Fellowship, held by 19 respondents (45.2%), followed by a Membership qualification (12 respondents, 28.6%), a Master's degree in Surgery (7 respondents, 16.7%), a Doctoral or research degree (2 respondents, 4.8%), and a primary medical degree only (2 respondents, 4.8%).

Primary Accreditation Body



COSECSA was the accrediting body for the largest share of respondents: 22 (52.4%) were COSECSA-accredited. National training programmes accredited 14 respondents (33.3%), international training programmes 3 (7.1%), WACS 2 (4.8%), and other bodies 1 (2.4%). The predominance of COSECSA accreditation aligns with the recruitment strategy and geographic concentration of respondents in East and Central Africa.

Years Since Specialty Accreditation



The vast majority of respondents — 30 of 41 with valid accreditation data (73.2%) — received specialty accreditation in 2020 or later, confirming the early-career profile of the sample. A further 5 respondents (12.2%) were accredited between 2010 and 2019, and 6 (14.6%) between 2000 and 2009. No respondents reported accreditation before 2000 (the one anomalous 1950 value was excluded as described in the data completeness section).

Professional Training Background

Table 2 summarizes professional qualifications, accreditation bodies, and periods of specialty accreditation among respondents. These findings characterize the training backgrounds and accreditation histories represented within the surveyed workforce.

Current Professional Position

Consultant/Attending surgeons were the most common position, comprising 16 respondents (38.1%). Residents and trainees accounted for 15 respondents (35.7%), making trainees collectively the second largest group. Academic faculty and Fellows each accounted for 4 respondents (9.5% each), and Department heads/chairs for 3 (7.1%). Taken together, the high proportion of trainees and early-career consultants (38.1% + 35.7% = 73.8%) reinforces the predominantly junior profile identified in the age and accreditation data.

Professional Training Background

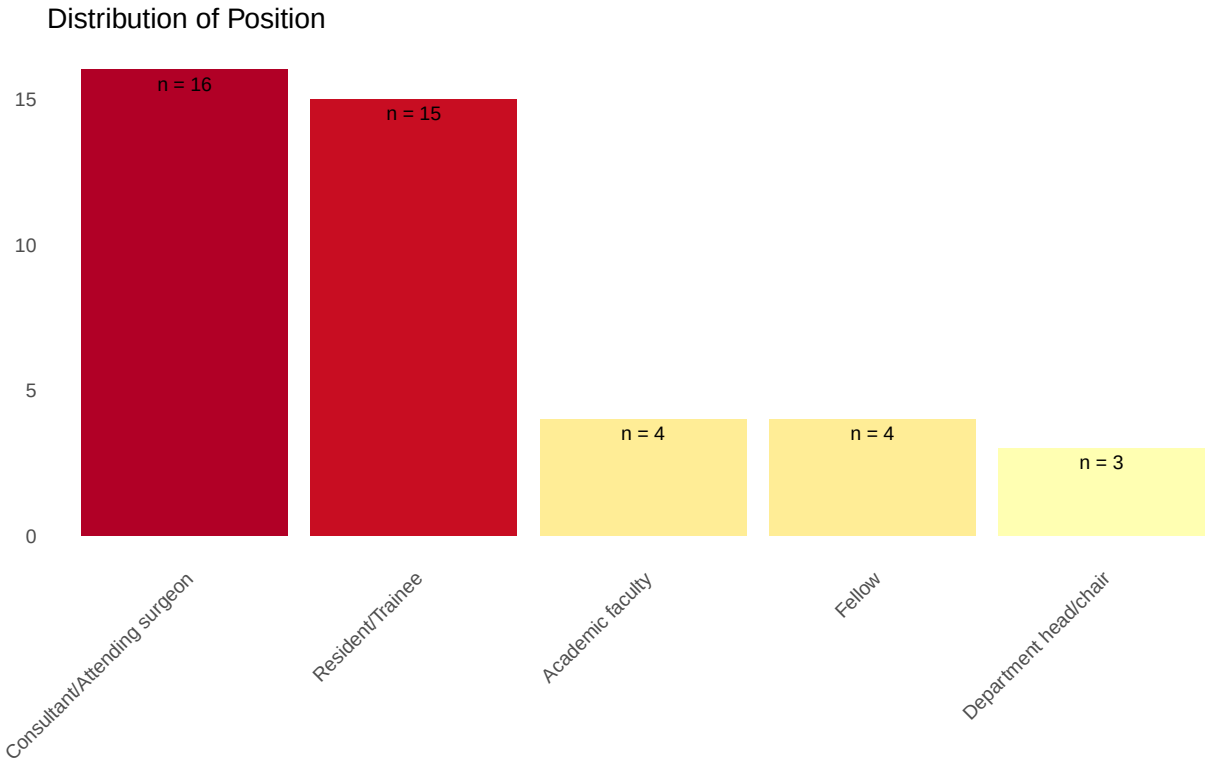
Total sample: N = 42

Characteristic	N	%
Highest Qualification	**42**	
Fellowship	19	45.2
Membership qualification	12	28.6
Master's degree in Surgery	7	16.7
Doctoral/research degree	2	4.8
Primary medical degree	2	4.8
Primary Accreditation Body	**42**	
COSECSA	22	52.4
National training program	14	33.3
International training program	3	7.1
WACS	2	4.8
Other	1	2.4
Year of Specialty Accreditation	**41**	
2000-2009	6	14.6
2010-2019	5	12.2
2020+	30	73.2

Current Professional Position

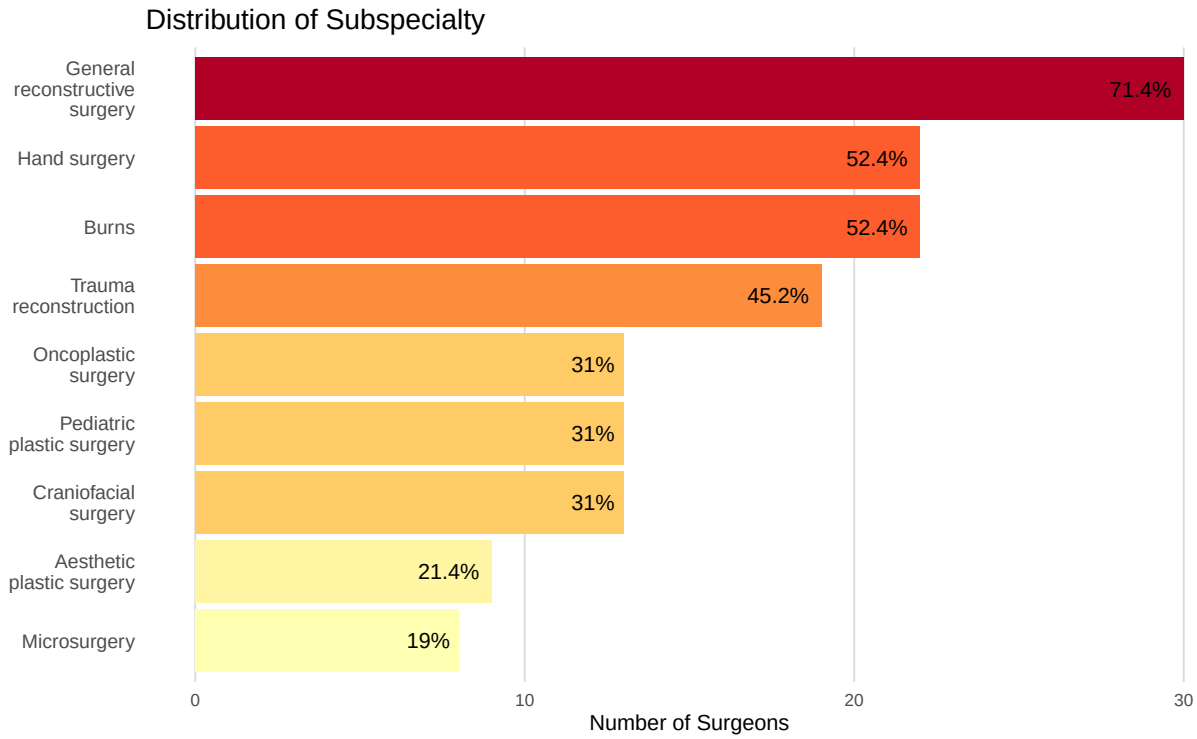
Total sample: N = 42

Professional Position	N	%
Academic faculty	4	9.5
Department head/chair	3	7.1
Consultant/Attending surgeon	16	38.1
Resident/Trainee	15	35.7
Fellow	4	9.5



Subspecialty

Respondents could select multiple subspecialties. Reported subspecialties spanned the breadth of plastic and reconstructive surgery, including burn surgery, hand surgery, craniofacial and cleft surgery, microsurgery, aesthetic surgery, and general reconstructive surgery, among others. Because respondents could identify with more than one area, percentages are not expected to total 100% and are reported by subspecialty in the accompanying figures.



Percentages represent the proportion of respondents selecting each subspecialty. Multiple selections permitted.

Practice Settings

Clinical practice settings were examined to characterize the institutions where respondents reported providing surgical services. The analysis considers facility characteristics, institutional ownership, geographic location, and the number of facilities associated with individual surgeons.

Because individual surgeons may practice at multiple institutions and multiple respondents may report the same facility, facility-level and respondent-level findings are presented separately.

Facilities Reported by Respondents

Survey respondents identified multiple healthcare facilities where they provided clinical services. The frequency with which each facility was reported is summarized in Table 4.

Respondents collectively identified 52 unique healthcare facilities across 12 countries. The University Teaching Hospital of Kigali (CHUK) was the most frequently reported facility, cited by 8 respondents, followed by ALERT Comprehensive Specialized Hospital in Ethiopia (5 respondents) and Yekatit 12 Hospital Medical College in Ethiopia (4 respondents). Rwanda Military Teaching and Referral Hospital was cited by 3 respondents; AIC Kijabe Hospital (Kenya), AaBET Hospital (Ethiopia), Felege Hiwot Hospital (Ethiopia), Mulago National Referral Hospital (Uganda), and Worabe Comprehensive Specialized Hospital (Ethiopia) were each cited by 2 respondents. The remaining 43 facilities were each cited by a single respondent.

Facility Characteristics

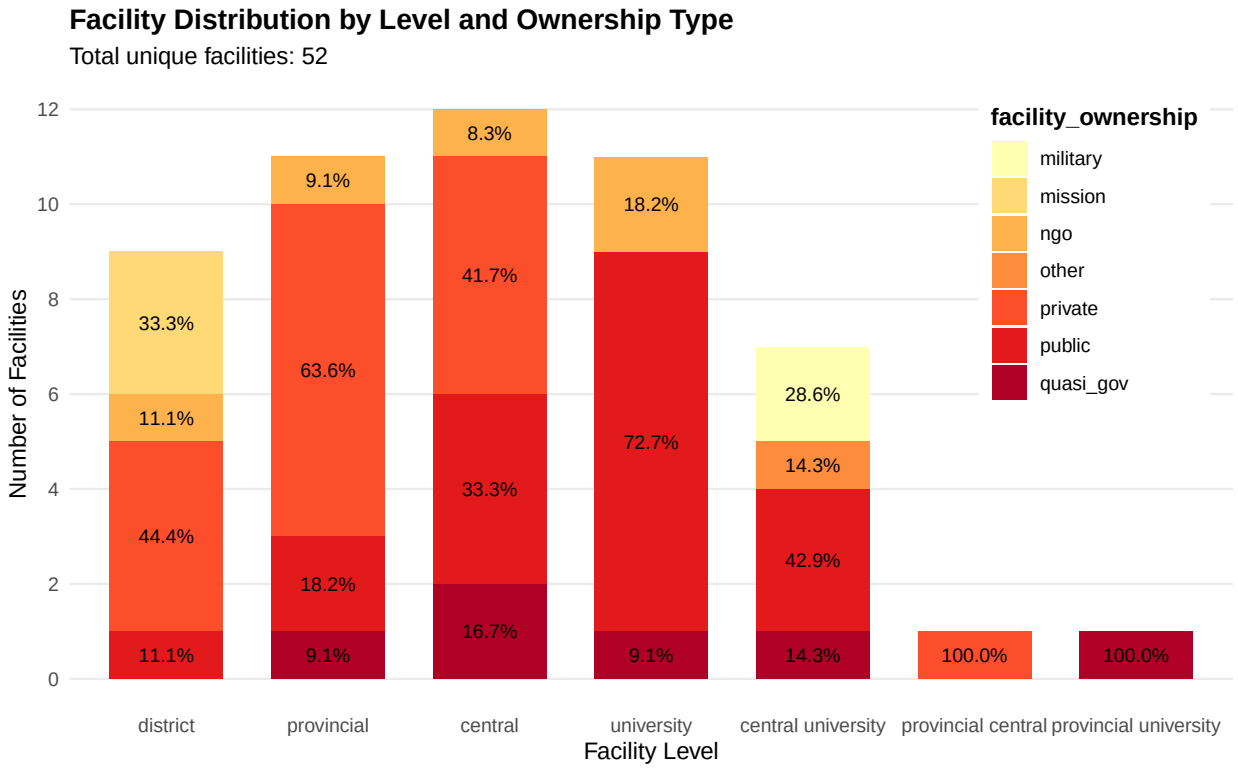
Reported healthcare facilities differed in geographic setting, institutional level, and ownership structure.

Table 5 summarizes the distribution of unique reported facilities across urban and rural settings, healthcare facility levels, and ownership categories. These characteristics provide context for understanding the institutional environments in which participating surgeons practice.

Facility Distribution by level and Ownership

The distribution of facility ownership across healthcare facility levels is presented in Figure 10.

This analysis examines the composition of reported institutions according to their level within the healthcare system and ownership structure, allowing comparison of institutional categories within the surveyed facilities.

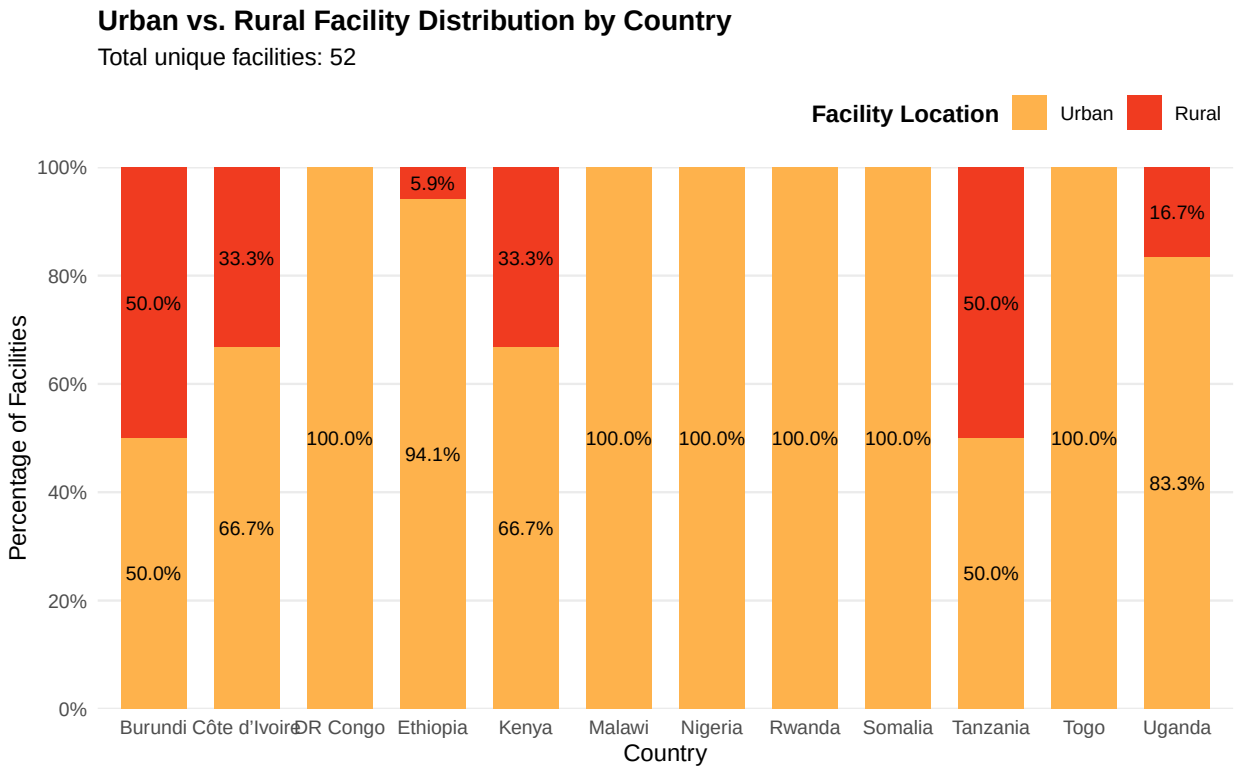


Reported facilities spanned multiple levels of the health system. Central hospitals accounted for 12 facilities (23.1%), provincial hospitals for 11 (21.2%), and university hospitals for 11 (21.2%). District hospitals contributed 9 facilities (17.3%), central-university combined hospitals 7 (13.5%), and provincial-central and provincial-university combined hospitals 1 each (1.9%).

Public facilities were the most common ownership type (18 facilities, 34.6%), closely followed by private facilities (17, 32.7%). Quasi-governmental facilities accounted for 6 (11.5%), NGO-operated facilities for 5 (9.6%), mission hospitals for 3 (5.8%), military hospitals for 2 (3.8%), and other ownership types for 1 (1.9%). The near-parity between public and private facilities signals that plastic surgery services in SSA are delivered through a mixed-ownership landscape.

Urban vs. Rural Facility Distribution by Country

The great majority of reported facilities were urban: 45 of 52 (86.5%), with only 7 (13.5%) classified as rural. This urban predominance was consistent across countries, suggesting that plastic surgery services are concentrated in city-based hospitals throughout the region.



Multiple Facilities

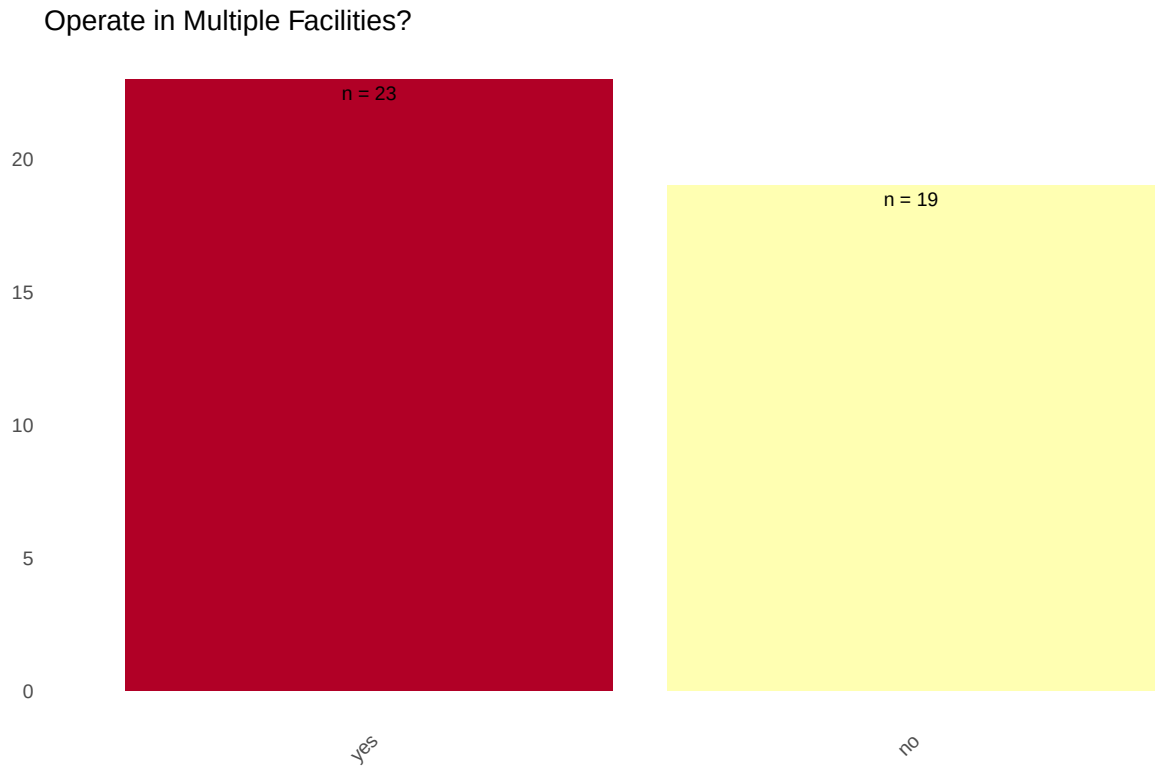
Survey respondents reported different patterns of institutional affiliation, including practice at a single facility and practice across multiple facilities.

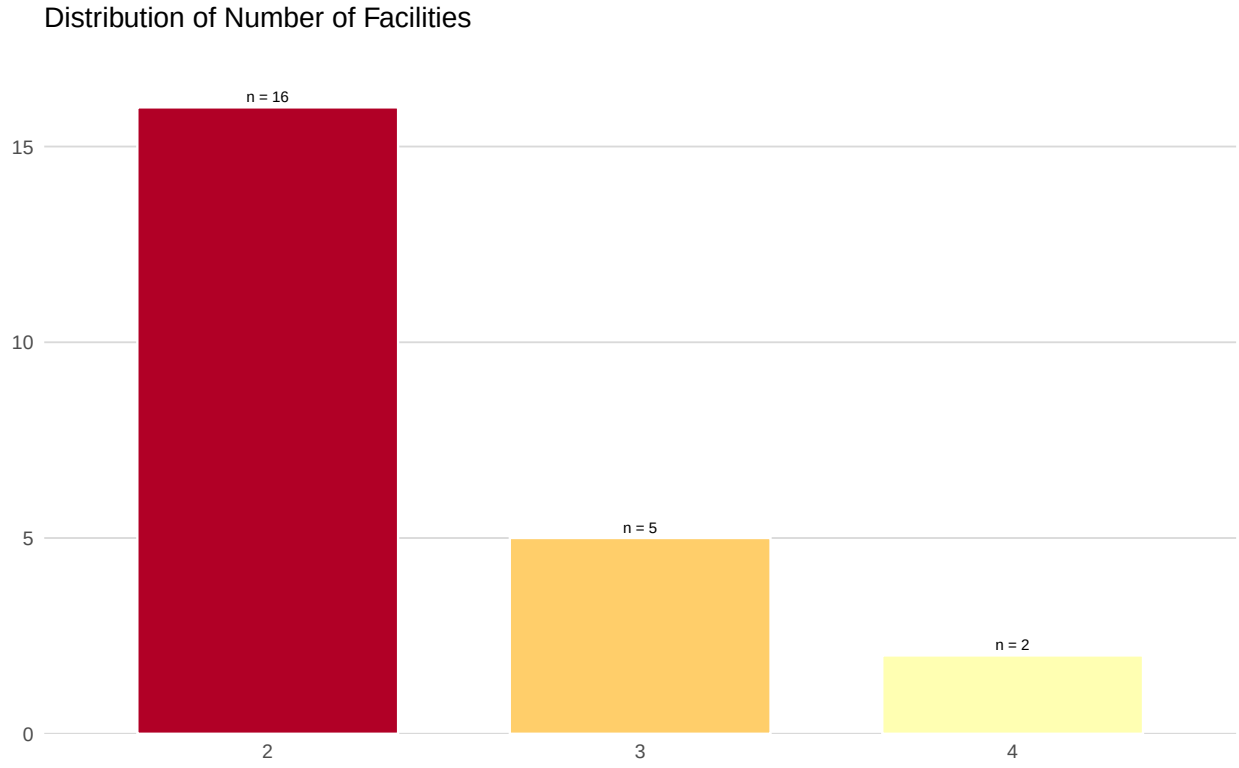
The proportion of respondents reporting employment or clinical activity at more than one facility is presented in Figure 12.

Number of Operating Facilities

The distribution of the number of facilities associated with individual respondents is presented in Figure 13.

These results describe the extent to which participating surgeons practice across multiple healthcare institutions and provide additional context for interpreting facility-level reporting within the survey.





A proportion of respondents reported practicing at more than one facility. The number of facilities per respondent is detailed in the accompanying figures. Practice across multiple institutions increases the aggregate service reach of individual surgeons but may also reflect fragmented or cross-subsidised service delivery — a pattern worth exploring in subsequent analyses.

Workforce Profile

A proportion of respondents reported practicing at more than one facility. The number of facilities per respondent is detailed in the accompanying figures. Practice across multiple institutions increases the aggregate service reach of individual surgeons but may also reflect fragmented or cross-subsidised service delivery — a pattern worth exploring in subsequent analyses. Workforce Profile and Staffing Respondents provided staffing estimates — plastic and reconstructive surgeons (PRS), PRS trainees, operating theatre (OT) nurses, and physiotherapists — for each of their reported facilities. Where multiple respondents reported the same facility, values are presented as median (minimum–maximum) to account for variation in individual estimates; single-respondent facilities report the value directly.

Staffing Numbers per Facility

Staffing numbers varied substantially across facilities and countries. Among the most heavily staffed facilities: - Saint Paul's Hospital Millennium Medical College (Ethiopia): 10 PRS surgeons, 17 PRS trainees, 20 OT nurses, 3 physiotherapists — the largest reported PRS surgeon count across all facilities. - Kenyatta National Hospital (Kenya): 5 PRS surgeons, 25 PRS trainees, 15 OT nurses, 4 physiotherapists — notable for the high trainee-to-surgeon ratio of 5:1. - King Faisal Hospital (Rwanda): 2 PRS surgeons, 25 PRS trainees, 10 OT nurses, 2 physiotherapists — the highest reported trainee count of any single facility. - ALERT Comprehensive Specialized Hospital (Ethiopia): median 8 PRS surgeons (range 7–9), 13 trainees (10–26), 24 OT nurses (16–30), 6 physiotherapists (5–10), based on 5 respondents. - Yekatit 12 Hospital Medical College (Ethiopia): median 5 PRS surgeons (5–7), 23 trainees (13–30), 12.5 OT nurses (10–15), 4.5 physiotherapists (3–6), based on 4 respondents. - University Teaching Hospital of Kigali — CHUK (Rwanda): median 1 PRS surgeon (1–4), 12.5 trainees (5–25), 3 OT nurses (2–10), 1 physiotherapist (0–2), based on 8 respondents — the most-reported facility, but with a notably low median PRS surgeon count.

Several facilities — including Nkinga Referral Hospital (Tanzania), Kalembelembe Pediatric Hospital (DR Congo), CHR Notsé (Togo), and Kalkaal Hospital (Somalia) — reported zero PRS surgeons, with either trainees only or minimal allied health support, suggesting severely limited specialist capacity. Staffing counts were not independently verified and represent self-reported estimates.

Pending Analyses

This report covers the demographic, professional training, geographic, and practice setting domains of the 2nd Chance Survey. Three survey domains have not yet been analysed and will be addressed in subsequent reports: service delivery, infrastructure and equipment, and training. Findings from these domains are expected to substantially enrich the workforce profile presented here and will be integrated into a comprehensive final report.

Facilities Reported by Respondents

N = 52 unique facilities

Facility Name	Respondents, n
University Teaching Hospital of Kigali (CHUK)	8
ALERT Comprehensive Specialized Hospital	5
Yekatit 12 Hospital Medical College	4
Rwanda Military Teaching and Referral Hospital	3
AIC Kijabe Hospital	2
AaBET Hospital	2
Felege Hiwot Hospital	2
Mulago National Referral Hospital	2
Worabe Comprehensive Specialized Hospital	2
AIC CURE International Hospital	1
Adama Hospital Medical College	1
Addis Hiwot Hospital	1
Avenue Hospital	1
Black Lion Hospital	1
CHR Notse	1
CHUSO	1
CORSU	1
Careland	1
Children's Surgical Hospital	1
Defence Forces Memorial Hospital	1
Dessie Hospital	1
Ethio Tebib Hospital	1
Gertrude's Children's Hospital	1
Grandville Medical and Laser Abuja	1
Grandville Medical and Laser Lekki	1
Grandville Medical and Laser Surulere	1
Grandville Trauma Center Lekki	1
Heal Venture	1
Hôpital Jean Baptiste Vatelot	1
Hôpital des Carmélites de Sakassou	1
Institut Raoul Follereau de Cote D'ivoire	1
Jimma University Medical Center	1
Kalembelembe Pediatric Hospital	1
Kalkaal Hospital	1
Kamuzu Central Hospital	1
Kayunga Regional Referral Hospital	1
Kenyatta National Hospital	1
King Faisal Hospital	1
Kiruddu National and Referral Hospital	1
Matana Referral Hospital	1
Meqrez Hospital	1
Muhimbili National Hospital	1
Nakasero Hospital Limited	1
Nkinga Referral Hospital	1
Polyclinelle	1
Queen Elizabeth Central Hospital	1
Saint Paul's Hospital Millennium Medical College	1
Tanganyika Hospital	1
University College Hospital, Ibadan	1
WoundSolutions Clinic	1
Yordanos Hospital	1
Yoya General Hospital	1

Facility Characteristics
N = 52 facilities

Urban vs. Rural, n (%)	**N = 52 facilities**
urban	45 (86.5%)
rural	7 (13.5%)
Level of Health Facility, n (%)	**N = 52 facilities**
central	12 (23.1%)
provincial	11 (21.2%)
university	11 (21.2%)
district	9 (17.3%)
central university	7 (13.5%)
provincial central	1 (1.9%)
provincial university	1 (1.9%)
Facility Ownership Type, n (%)	**N = 52 facilities**
public	18 (34.6%)
private	17 (32.7%)
quasi_gov	6 (11.5%)
ngo	5 (9.6%)
mission	3 (5.8%)
military	2 (3.8%)
other	1 (1.9%)

Reported Surgical Workforce by Facility and Country

Facility-level workforce estimates

Facility	Respondents (n)	PRS surgeons	PRS trainees	OT nurses	Physiotherapi
Burundi					
Matana Referral Hospital	1	1	0	1	1
Tanganyika Hospital	1	2	1	2	0
Côte d'Ivoire					
Hôpital Jean Baptiste Vatelot	1	3	4	19	2
Hôpital des Carmélites de Sakassou	1	3	2	8	1
Institut Raoul Follereau de Cote D'ivoire	1	2	5	2	7
DR Congo					
Kalembelembe Pediatric Hospital	1	0	1	8	0
Ethiopia					
ALERT Comprehensive Specialized Hospital	5	8 (7–9)	13 (10–26)	24 (16–30)	6 (5–10)
AaBET Hospital	2	7 (4–10)	11.5 (10–13)	15	4.5 (2–7)
Adama Hospital Medical College	1	2	0	7	3
Addis Hiwot Hospital	1	2	0	12	2
Black Lion Hospital	1	5	15	4	15
Careland	1	1	0	2	2
Dessie Hospital	1	2	0	6	1
Ethio Tebib Hospital	1	1	0	5	4
Felege Hiwot Hospital	2	2	0	8	4
Heal Venture	1	1	0	4	0
Jimma University Medical Center	1	3	0	9	2
Meqrez Hospital	1	3	0	8	5
Saint Paul's Hospital Millennium Medical College	1	10	17	20	3
Worabe Comprehensive Specialized Hospital	2	2.5 (2–3)	0.5 (0–1)	7.5 (5–10)	1.5 (1–2)
Yekatit 12 Hospital Medical College	4	5 (5–7)	23 (13–30)	12.5 (10–15)	4.5 (3–6)
Yordanos Hospital	1	2	0	10	3
Yoya General Hospital	1	2	0	9	2
Kenya					
AIC CURE International Hospital	1	2	0	6	10
AIC Kijabe Hospital	2	3	5	4	7.5 (5–10)
Avenue Hospital	1	4	0	4	2
Defence Forces Memorial Hospital	1	1	0	0	1
Gertrude's Children's Hospital	1	3	0	10	10
Kenyatta National Hospital	1	5	25	15	4
Malawi					
Kamuzu Central Hospital	1	2	0	8	2
Queen Elizabeth Central Hospital	1	3	3	30	4
Nigeria					
Grandville Medical and Laser Abuja	1	2	1	6	1
Grandville Medical and Laser Lekki	1	1	0	6	2
Grandville Medical and Laser Surulere	1	3	0	8	1
Grandville Trauma Center Lekki	1	3	0	10	2
University College Hospital, Ibadan	1	4	4	3	3
WoundSolutions Clinic	1	1	0	1	0
Rwanda					
King Faisal Hospital	1	2	25	10	2
Rwanda Military Teaching and Referral Hospital	3	2 (2–3)	5 (4–25)	10 (2–12)	2 (1–5)
University Teaching Hospital of Kigali (CHUK)	8	1 (1–4)	12.5 (5–25)	3 (2–10)	1 (0–2)
Somalia					
Kalkaal Hospital	1	1	0	0	1
Tanzania					
Muhimbili National Hospital	1	7	6	2	2